MERCY'S ANATOMY

Series Bible, Document MA-001: The Foundation

Canon Lock v1.0. Story present: 2005. Setting: Long Island City, Queens, New York

STATUS: LOCKED IN. Everything in this document is now canon for *Mercy's Anatomy*. Later documents can only add to it. Changing anything here requires an explicit amendment that names the section it replaces. Characters and episodes are **reserved for the creator** and will be added in later documents. This document lays the foundation they will sit on.

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1. Locked Rules (The Canon Charter)

#	Rule	What it means in practice
1	The present is 2005	The story begins in 2005 and moves forward in real chronological order. Nothing anachronistic is allowed: no technology, slang, prices, laws or events from after the in-story date.
2	Equivalent to Grey's Anatomy but original	It matches Grey's in format, tone, scope, structure and emotional DNA: an ensemble of surgical interns, residents and attendings, their personal and professional lives intertwined, cases of the week, big event episodes and long-running romance. Every character, name, storyline, line of dialogue and case is original. Nothing is copied.
3	PDFs only	Every deliverable for this project is produced as a PDF.
4	This is the story	There is no separate behind-the-scenes, real-world production or cast story. The documents ARE the world of Mercy's Anatomy.
5	Multi-format storytelling	Told through news articles and headlines, social media and the internet (all accurate to 2005), plus a lot of character-driven dialogue in screenplay format.
6	Real-life realism	Medicine, hospital politics, New York City geography, law, money and human behavior all follow how things really worked in 2005.
7	Dated timeline	Every scene, article and post has a date and a time. Every character's age is calculated from their birthday.
8	Continuity is sacred	Established facts never contradict each other. See Section 9.
9	The No-Nerf Rule	Absolutely nobody gets nerfed. A surgeon established as elite stays elite. Nobody's skills, intelligence, toughness or backstory get quietly reduced to make a plot work. Characters can fail, but only for reasons the story has earned, never because the writing needs them to.
10	Source-faithful	Once something becomes canon in our documents, it is the source material. Later documents must stay 100% faithful to it.
11	Creator's reserve	Characters and episodes are created by the creator. Nothing in this bible invents them.

2. Serialized vs. Standalone: The Perfect Balance

THE RULING: 65% serialized / 35% standalone.
The serialized 65% covers character arcs, romances, rivalries, careers, hospital politics and season mythology. The standalone 35% covers the patient cases of the week, which open and close inside a single episode.

Why 65/35 is the right number. A show built on the Grey's model succeeds because viewers tune in for the *people* and stay for the *cases*. If you go more serialized than about 70%, a casual viewer who misses a week is lost, and the hospital starts to feel like a backdrop rather than a pressure cooker. If you go more standalone than about 40%, it becomes a procedural, and the romance, rivalry and ambition that drive a medical soap stop paying off. At 65/35, every episode satisfies on its own, but you can't

stop watching.

Weekly episode architecture

Story strand	Share of episode	Type	Function
A-Story: lead character's personal or professional arc	~35%	Serialized	Moves the season engine forward
B-Story: the main patient case	~25%	Standalone	Resolves this week and mirrors the A-Story's theme
C-Story: ensemble arcs (romance, rivalry, family)	~25%	Serialized	Keeps every cast member moving
D-Story: secondary cases and hospital texture	~10%	Standalone	Humor, realism, quick wins or losses
Voiceover bookends (open and close)	~5%	Thematic	Sets the episode's question, then answers it

Season-level rhythm

- **Event episodes** (mass casualty, disaster, lockdown, multi-part crises) are the most serialized, at 85-90%. Use them at midseason and in the finale.
- **Breather episodes** (one long shift, one big case) can drop to about 50/50, but they must still advance at least two arcs.
- **A patient can return** later in the series only if continuity supports it: same name, same history, the right age for the new date.
- **Every season** needs a mythology: a central question such as a merger, a scandal or a chief's job that pays off in the finale.

3. Genre, Subgenres, Rating and Runtime

Category	Ruling
Primary genre	**Medical drama**
Core subgenres	Workplace ensemble drama; romantic drama (prime-time soap); coming-of-age / professional bildungsroman (interns growing into surgeons)
Supporting subgenres	Dramedy (sharp, character-based humor that breaks up the tension); disaster or event drama (mass casualty episodes); social-issue drama (insurance, immigration, race, gender and sexuality in medicine, as they stood in 2005 Queens); family drama
Tone	About 70% drama and 30% wit. Grounded, emotional, sexy, smart and sometimes devastating. Real stakes, real grief, real joy.
Rating	**TV-14** as the baseline, with episode descriptors as needed: **TV-14-DLSV** (suggestive dialogue, language, sexual situations, violence). Surgical gore is realistic but not exploitative.

Category	Ruling
Runtime	**One-hour drama**: about **43 minutes** of content per episode (a 60-minute broadcast slot with commercials, which is the 2005 standard). Premieres and finales can run as two-hour or 'super-sized' events of about 60-86 minutes.
Act structure	Cold open (teaser), then 5 acts, then a tag. Each act ends on a hook built to carry viewers through a 2005 commercial break.
Season length	A full broadcast season is 22-24 episodes. A shortened launch season (as with many 2005 midseason premieres) is also possible. Final episode counts are reserved for the creator.
Signature device	Opening and closing voiceover from the lead character, which frames the episode's theme. The original form and voice are reserved for the creator.

4. Premise

Mercy's Anatomy follows the personal and professional lives of surgical interns, residents and attendings at **Queens Grace Hospital**, a Level I trauma center and teaching hospital in Long Island City, Queens. Across eighty-hour weeks, thirty-hour shifts and life-or-death decisions, a new class of interns tries to survive the most competitive surgical program in the outer boroughs. Meanwhile, the attendings above them fight their own wars over power, love, ego and legacy. In Queens Grace, the line between saving a life and wrecking your own is one scalpel wide. Over the years, the hospital itself changes: it is renamed **Queens Grace Hudson Mercy Hospital** and finally **Bullock Sloan Memorial Hospital**.

5. Setting: Long Island City, Queens (2005)

In 2005, Long Island City is a neighborhood in transition. It still has old warehouses, factories and gritty industrial blocks, but luxury high-rises are starting to go up on the waterfront after the 2001 rezoning. It looks straight across the East River at Midtown Manhattan. That tension (old Queens against new money, the working class against gentrification) is built into the show's DNA.

Real 2005 landmarks for texture

- **Citigroup Building (One Court Square)**: the tallest building in Queens and the skyline anchor.
- **The Pepsi-Cola sign** and **Gantry Plaza State Park** on the waterfront, with Manhattan skyline views. It works as a natural spot for late-night confessions.
- **5Pointz**: the graffiti mecca on Jackson Avenue and Davis Street, still active in 2005.
- **Silvercup Studios**, the **Queensboro (59th Street) Bridge**, **Vernon Boulevard** diners and bars, and **Newtown Creek**.
- **Subway**: the 7 train (Vernon Blvd-Jackson Av, Hunters Point Av, 45 Rd-Court House Sq), the E and V (23 St-Ely Av), and the G at Court Square. The MetroCard costs $2.00 per ride in 2005.
- **Nearby**: Astoria, Sunnyside, Woodside, Greenpoint (Brooklyn, across Newtown Creek), and Midtown one stop away.

Recommended hospital location (open to creator amendment)

A full city block near **Jackson Avenue and Court Square**, in the shadow of the Citigroup tower. It sits within ambulance reach of the Queensboro Bridge, the Long Island Expressway and the Queens-Midtown Tunnel, which makes it a logical receiving trauma center for bridge, highway, subway and waterfront incidents. It also has a rooftop with a direct view of the Manhattan skyline.

6. The Hospital: Three Names, One Building

Era	Name	Story function
Era I (from 2005)	**Queens Grace Hospital**	The original identity: a proud, underfunded, scrappy teaching hospital and Level I trauma center serving Queens.
Era II	**Queens Grace Hudson Mercy Hospital**	The merger era. Queens Grace combines with Hudson Mercy, bringing culture clashes, new staff, layoffs, power struggles and divided loyalties. The dates and circumstances are reserved for the creator.
Era III	**Bullock Sloan Memorial Hospital**	The memorial era. The name honors the legacy of figures named Bullock and Sloan. Who they were, and the events behind the renaming, are reserved for the creator.

Continuity rule: every in-story document uses the hospital name that was correct on its date. A 2005 newspaper never says 'Hudson Mercy.' After a renaming, characters may still use the old name in dialogue, which is realistic, but official signage, press coverage and paperwork switch to the new one.

Department and hierarchy (2005 teaching-hospital model)

Chief of Surgery, then Department Heads / Attending Surgeons, then Chief Resident, then Residents (PGY-2 to PGY-5), then Interns (PGY-1). Supporting staff include nurses (charge nurses, OR and ER nurses), the Chief of Staff, the hospital board and administration, paramedics (FDNY EMS) and hospital security.

7. Storytelling Formats and the 2005 Media Rulebook

Format	2005-accurate options	Off-limits (anachronistic)
Newspapers / print	New York Daily News, New York Post, The New York Times, Newsday, the Queens Chronicle, the Queens TimesLedger, and Western Queens Gazette-style community papers	None
TV / radio news	NY1, WABC-7 Eyewitness News, WNBC-4, WCBS-2, FOX 5, 1010 WINS radio	Live-streams
Internet	News websites, blogs (Blogger, LiveJournal, Xanga), message boards, AOL, Yahoo!, Google, Wikipedia (young), Craigslist, and patient or medical forums	Reddit (launched June 2005; only niche use allowed after that), Twitter / X (2006), Instagram (2010)
Social media	MySpace (dominant), Friendster, Facebook (college students only in 2005; it opens to the public in Sept 2006), YouTube (launched 2005), Flickr	Facebook for adults before Sept 2006, smartphone apps
Messaging	AIM (away messages!), MSN Messenger, Yahoo! Messenger, SMS on flip phones (Motorola RAZR, Nokia), BlackBerry email for attendings and administrators, hospital pagers	iPhone (2007), emoji, FaceTime
Screenplay scenes	Character-driven dialogue in industry-standard format (Section 11)	None

Every non-screenplay item carries a source, date and time stamp, e.g. *NEW YORK DAILY NEWS, Thursday, March 31, 2005, 6:00 AM edition*, or *MySpace bulletin posted Sat 04/02/2005 2:17 AM*.

8. Timeline, Date and Age Protocol

- **Every scene heading carries a date and a time**, e.g. *MONDAY, MARCH 28, 2005, 5:42 AM*. Real 2005 calendar days of the week are used (for example, January 1, 2005 was a Saturday).
- **Every character has a fixed date of birth** on their character sheet, assigned by the creator.
- **Ages are calculated, never estimated**: age = (current year minus birth year), minus 1 if the birthday hasn't happened yet that year. For example, someone born on November 14, 1976 is 28 on March 28, 2005 and turns 29 on November 14, 2005.
- **Birthdays are trackable events.** A character's age changes on screen on the correct date, and birthday episodes have to land on the right day.
- **Real-world 2005 context is honored** where it fits, without being exploited: weather, holidays, the NYC transit strike (Dec 20-22, 2005), the Yankees/Mets seasons, the November 2005 NYC mayoral election and so on.

- **Time moves forward only.** Flashbacks are allowed but are clearly dated.

9. Continuity System

Continuity is **extremely important** for *Mercy's Anatomy*. These are the tools that protect it:

Tool	Purpose
Document numbering (MA-###)	Every PDF gets a permanent sequential number. This is MA-001.
Canon Log	A running list of every established fact: who slept with whom, who knows what secret, injuries, scars, deaths, job titles, addresses, cars, and family members.
Character Sheets	DOB, age at series start, hometown, education (college, medical school, match year), residency year, specialty, family, relationships and medical history.
Master Timeline	Every dated event in chronological order, across all formats (episodes, articles and posts).
Patient Registry	Every named patient: their name, DOB, case, outcome and surgeon. Used for callbacks and for avoiding duplicates.
Knowledge Tracker	Who knows each secret, and since when. A character can never act on information they don't have yet.
Hospital Status Log	The current name, chief, board, funding state and trauma designation as of each date.

10. Medical Realism Standards (2005)

- **Duty hours:** the ACGME 80-hour work week, maximum 30-hour shifts (24 hours plus 6 for handoffs) and one day off in seven. These rules were enforced from 2003 and are a live source of conflict in 2005.
- **Residency** is a 5-year general surgery program, plus research years or fellowships. The Match takes place in March. The academic year turns over July 1 ('July effect').
- **2005 technology:** paper charts, pagers, film or early PACS imaging, early robotic surgery (da Vinci), laparoscopy and the start of the electronic records rollout.
- **Law and ethics:** HIPAA (in force since 2003), informed consent, M&M conferences, malpractice and New York State Department of Health oversight.
- **Cases** have to be medically plausible: real anatomy, real procedures, real complication rates and realistic outcomes.

11. Screenplay Format Standard (with sample)

Dialogue scenes use industry-standard screenplay format: scene heading, then date/time stamp, then action, then character cue, then parenthetical, then dialogue. *The sample below shows FORMAT ONLY. It uses placeholder labels instead of real characters, which are reserved for the creator.*

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INT. QUEENS GRACE HOSPITAL - ATTENDINGS' LOUNGE - NIGHT
MONDAY, MARCH 28, 2005, 11:58 PM
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Rain streaks the window. Beyond it, the Pepsi-Cola sign glows red across the river from the Manhattan skyline. A pager buzzes on the table.

[ATTENDING A]
(not looking up)
If that's the ER, I'm not here.

[RESIDENT B]
It's the bridge. Six-car pileup on the Queensboro. Four criticals, ten minutes out.

[ATTENDING A] stands, already tying back their hair.

[ATTENDING A]
Then I'm here.

SMASH CUT TO:

12. Open Slots Reserved for the Creator

- All characters: names, birthdays, backgrounds, relationships and arcs.
- All episodes: titles, dates, plots and cases.
- The series premiere date (somewhere in 2005).
- The Hudson Mercy merger: when, why and how.
- Who Bullock and Sloan are, and why the hospital is named for them.
- The voice and form of the lead's voiceover.
- Season counts and episode orders.

END OF MA-001. CANON LOCKED.
The foundation is poured. The OR is prepped. Bring the characters and we'll scrub in.